

Question:

A mental health volunteer is interviewing locals as part of a community outreach program. A 46-year-old man discloses that he has felt sad for as long as he can remember. He feels as though his life is cursed and if something terrible can happen to him, it usually will. He has difficulty making decisions and feels hopeless. He also feels that he has had worsening suicidal ideations, guilt from past problems, energy, and concentration over the past 2 weeks. He is otherwise getting enough sleep and able to hold a job. Which of the following statement best describes this patient's condition?

Answer Choices:

- A. The patient is at risk for double depression.
- B. The patient may have symptoms of mania or psychosis.
- C. The patient is likely to show anhedonia.
- D. The patient likely has paranoid personality disorder.

Reasoning + Answer:

<think>

Finding Reasoning Paths :

1. **Feelings of sadness , hopelessness , and belief that terrible things will happen** â Depression .

2. **Difficulty making decisions and poor concentration** â Cognitive dysfunction , potentially related to depression or another mood disorder .

3. **History of past guilt and worsening suicidal ideations** â Major depressive disorder (MDD) or suicidal ideation .

4. **Reported by a mental health volunteer interviewing locals** â Mental health assessment and diagnosis .

5. **Current symptoms over the past 2 weeks** â Acute symptom presentation , likely reflecting a recent episode of a chronic condition like depression or a new-onset disorder .

6. **Sleep and job stability** â Absence of major impairments in daily functioning , which may influence the classification of symptoms .

Reasoning Process :

1. **Evaluating depressive symptoms**: The patient's long-standing history of sadness , hopelessness , and the belief that " terrible things will happen " are classic depressive symptoms . The worsening of suicidal ideations and past guilt further support the presence of depressive symptoms . These symptoms suggest major depressive disorder (MDD), which is consistent with the presentation of long-term mood disturbances and acute exacerbation .

2. **Assessing decision-making and concentration**: The difficulty in making decisions and poor concentration could be due to depression , as this disorder often causes cognitive dysfunction . However , these symptoms are also associated with other conditions such as bipolar disorder or schizophrenia . Given the absence of manic symptoms (e.g ., elevated mood , increased energy) and psychotic features , MDD remains the most likely diagnosis .

3. **Analyzing the risk for double depression**: Double depression is a term used to describe recurrent major depressive episodes on the same episode of MDD . This fits the patient's history of long-standing sadness and recent acute exacerbation , making it a plausible diagnosis .

4. **Evaluating other potential diagnoses**:

- **Bipolar disorder**: The absence of manic symptoms makes this less likely .

- **Schizophrenia**: The lack of psychotic symptoms (e.g ., hallucinations , delusions) makes this highly unlikely .

- **Generalized anxiety disorder (GAD)**: While the patient may experience anxiety , the primary presentation is depressive rather than anxious .

- **Personality disorders**: While the patient's feeling of being " cursed " could be seen as suspicious , the acute nature of symptoms and the absence of pervasive personality traits make personality disorders unlikely .

5. **Considering the role of sleep and work**: The absence of major impairments in daily functioning and the ability to hold a job do not rule out depression but may suggest that the symptoms are not as severe as in a more debilitating form of the condition .

Conclusion :

The patient's condition is best described as a **risk for double depression** . The long-standing history of depressive symptoms , recent acute exacerbation , and difficulty with decision-making and concentration are consistent with this diagnosis . The absence of major impairments in daily functioning does not significantly alter this conclusion .

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<answer>

The patient is at risk for double depression .

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